

The Registrar
Victoria Hospital,
Victoria Rd.,
Melbourne

Date: 22 Feb. 2002

Re: Mrs Fiona Marsden

Dear Dr,

This 30 year-old, mother of 2, housewife, came to my surgery 2 days ago, complaining of mild abdominal pain for a few days. She claimed that her pain was in the lower abdominal area and could be relieved by panadol; however, she denied nausea or vomiting.

Her physical examinations were normal, except a mild tenderness over the right abdominal quadrant with no evidence of hepatosplenomegaly. The possible diagnosis was ovarian cyst enlargement; therefore, FBE, ESR tests and a pelvic ultrasound were requested with a revisit arranged for one week's time.

Mrs Marsden visited my office today as an emergency appointment, complaining that her pain has gradually become worse and severe in intensity since few hours ago. She also reports being weak and fatigued since then.

On examination, she is pale and restless; moreover, there is a severe tenderness with palpable mass in the lower right abdominal quadrant. Based on the evidence, there is a strong possibility that she is suffering from a ruptured ovarian cyst.

Please note that she had a history of tubular ligation two years ago, and a right ovarian cyst (5-6cm) 6 months ago when she was put on 3 months of OCP.

I am referring her to you for admission to your gynaecological ward for further investigation and management regarding her urgent problem.

Thanking you in advance,

Yours sincerely,

Dr X----- (GP)

UNSWIL Medical Centre
22 King Street
Randwick NSW 2031

24/08/2002

Dr. G Brian
Emergency Department Medical Officer
Alfred Hospital
Commercial Road
Prahan Victoria.

Dear Dr. Brian,

Thank you for seeing Mr. Johnson, a 25 year old man suffering from severe abdominal pain.

He first came to see me at 10am yesterday complaining of crampy central abdominal pain, nausea and several loose bowel movements which he had had in the previous 24 hours. On examination I found that he had generalized central abdominal tenderness. All other investigations were normal. I prescribed analgesics and advised him to increase his fluid intake and to rest.

At 9.00am today, Mr. Johnson presented again as his condition had deteriorated. His pain had increased and had vomited several times overnight. He was also slightly flushed and had tenderness on his right side with mild guarding. Other test results were normal. I suspected viral infection and possibly appendicitis and ordered a blood film. I prescribed Maxolon 10gm four hourly for the vomiting.

Mr. Johnson has come to see me again as his condition has worsened. The pain is now severe, constant and localized to right iliac fossa. He is flushed and restless. The blood film shows a white cell count of 18,000. I suspect that he has acute appendicitis.

I would appreciate if you could urgently assess Mr. Johnson and treat him further.

Yours sincerely

J Harris

Dr J Harris

Dr Michaels
St. Patrick Ave,
Melbourne

Date: 30th June 2004

Re: Mrs Patricia Gordon

Dear Dr Michaels,

I am referring Mrs. Patricia Gordon who first visited my surgery two years ago. She had just married and requested contraception pills; however, she did inform me of her intention to have a family in one to two years time.

On examination at the time, all the test results were unremarkable – breast examination, pap- smear, Rubella test. She was subsequently prescribed Microgynon 30 ED 12x12.

On her next visit last year, she reported that she had stopped her contraceptive pills two months earlier, with the intention of getting pregnant but her menstrual cycle had started again. She was rather disappointed at the time that she did not get pregnant. We discussed temperature and cycle measurements to augment her chances of conception.

She visited my office two weeks ago, tearful and depressed, as nothing seemed to have worked. The temperature chart showed definite and appropriate changes. Her pelvic and general examinations were unremarkable as was her pap-smear test. She was asked to return with her spouse a week later for further investigation.

Last week, Mr. Gordon's examination revealed a healthy young man, and consequently a sperm-count test was arranged.

This morning the couple returned for the result of the test, which showed normal count and viable sperms.

As this couple is keen to start a family soon; I am, therefore, referring this couple to you for further investigation, diagnosis and advice.

Thanking you in advance,

Yours sincerely,

Dr X-----

Doctors – Letter of Referral

Dr F Goldman
171 Victoria Parade
East Melbourne

30/01/2005

Re: Jamie Brown
DOB: 10.5.85

Dear Dr Goldman,

This 5 year-old boy Jamie Brown initially presented with tonsillitis on the 20/12/2005 for which he was treated with Penicillin V 250mg qid for 7 days.

Four weeks later Jamie presented with painless macroscopic haematuria. His only other symptom appeared to be lethargy. Examination was unremarkable: he had tonsillar hypertrophy, his BP – 90/60 and urinalysis confirmed significant haematuria.

On review today, investigations showed a mildly elevated urea and creatinine, a significantly elevated ASCOT titre and haematuria of renal origin.

Patient's blood pressure has increased to 110/90.

I believe Jamie has post-streptococcal nephritis and that he is at risk of developing renal failure. I would appreciate your assessment and management.

Yours sincerely,

Dr X

Mr. Lawrence Mitchell, surgeon.
Suite 12,
Cabrini Medical Centre,
Malvern 3127

2nd Feb 2005

Re: Mr Patrick Freeman D.O.B 20.01.57
16 Garden Ave
Brisbane
QLD
Tel: 0413 111 333

Dear Dr Mitchell,

I am writing to refer Mr Patrick Freeman, a 48-year-old sales manager, to you for further management. He presented at my surgery one month ago complaining of pain in the epigastric area which was accompanied by vomiting and tiredness for the past few months. However, there was no Melena at this point and his pain was relieved by eating a meal.

Duodenal ulcer was a possible diagnosis; therefore, a full set of tests were requested which included X-ray Barium meal.

He returned 3 days later for his results. His pain, nausea and vomiting still persisted. His FBE was normal, but there was a slight increase in ALAT. Also, there was no evidence of ulcer in the X-ray. He was advised to improve his diet and give up both cigarettes and coffee. In addition, Tagamet (200mg tid) was prescribed and he was asked to revisit in 6 weeks time.

He, however, came to my surgery today (3 weeks early), complaining of the epigastric pain and black faeces for one week. On examination, he was pale, agitated and preferred to keep his posture upright. The upper abdominal quadrant was tender; although, his bowel sounds were normal.

Please note that Mr Freeman was hypotensive (B.P 80/50; P 110/min) this morning. He is a 2 pack a day smoker and drinks 5-6 cups of coffee per day.

I believe he is suffering from a bleeding duodenal ulcer which requires immediate attention. I would appreciate if you could examine, assess and treat this patient as you think appropriate.

Thank you in advance,

Yours sincerely,

Dr -----

Dr Peter Groves
Paediatric gastroenterologist
Royal Children's Hospital

Date: 21st February 2005

Re: Master Ben Schmidt

Dear Dr Groves,

I am writing to refer Master Ben Schmidt, an 8 year old patient of mine, who was first brought to my office by his parents, David and Sue, 12 days ago. On examination, he was feverish (Temp. 40.5 °C) with flu and exhibited symptoms relating to Asthma. Otitis media was also diagnosed; therefore, a course of Amoxicillin was prescribed. In addition, Paracetamol tablets as well as Ventolin (2 puff q.i.d) were included.

Ben was brought back to my surgery five days later by his mother on 14th Feb, complaining of his son vomiting. The examination revealed a lowering of his temperature (39.5) but the Asthma was consistent. At this point only Maxilon injection was prescribed.

Yesterday, his mother brought him to my office again this time complaining of bleeding from his bowels which apparently had started 3 days prior. His previous problems had settled, although he was feeling lethargic due to his flu infection. The physical examination revealed no anal fissure. A full set of blood tests were requested at this point to aid with the diagnosis.

Mrs Schmidt was asked to come to my surgery today with Ben. The results show that he is anemic (Hb - 7), and since the problem still persists, I believe this is a serious problem and calls for a colonoscopy and further internal examination to detect the source and cause of hemorrhage.

I would appreciate if you could examine, diagnose and treat this patient as you think appropriate,

Thank you in advance,

Yours sincerely,

Dr X-----

Dr F Goldman
171 Victoria Parade,
East Melbourne

Date: 30 Jan 2005

Re: Jamie Brown

Dear Dr Goldman,

I would like to refer Master Jamie Brown, a five-year-old boy, who was brought by his mother to my surgery six weeks ago. He had sore throat at the time, with a husky voice, feverish and irritable.

On examination, large infected tonsils with exudates were observed as well as tender and enlarged cervical lymph nodes. Accordingly, tonsillitis was diagnosed and a course of penicillin V250 mg (q.i.d) was prescribed for 7 days.

On his return two days ago, his mother reported Jamie to be tired, lethargic and had passed brownish urine 4 days earlier. On examination, he was hypotensive (BP 90/60), had tonsillar hypertrophy, and his urinalysis revealed macroscopic haematuria. With possible diagnosis of UTI or post-streptococcal nephritis, prescribed a number of tests. Plenty of fluid intakes were recommended and a revisit was arranged for two days time.

Jamie was brought to my surgery today by his mother, still asymptomatic. However, elevated blood pressure was observed. His test results showed elevated urea, creatinine and ASCOT. In addition, his urinalysis indicated a macroscopic haematuria, and his mid-stream urine test revealed 4×10^6 RBC, attributed to renal origin.

I would like to refer Jamie Brown to you with a diagnosis of post streptococcal nephritis with early renal failure. Could you please assess, investigate and treat him as you think appropriate.

Thank you in advance,

Yours sincerely,

Dr X------(GP)

Dr. William Jones
The medical centre
46 Prince Street
AUBURN NSW 2247

Date: 28 January 2006

Re: Mrs. Trudy McHugh
7 Alfred Street
Sydney NSW 2000

Dear Dr Jones,

This 38-year-old woman came to my surgery yesterday, complaining of a sudden onset of left lower abdominal pain for one day. She claimed that it was sharp and constant which worsened by sitting up, walking or bending. There were no other symptoms; however, she claimed that she had another episode of left abdominal pain one week before.

On examination, there was a great tenderness on the left quadrant of her abdomen, and a vague mass was also palpated in the same region. To exclude pregnancy, β -HCG was requested; in addition, full blood examination and ESR were checked too.

On her visit today, the pain still persisted, but it has become easier. There have not been any bowel motions since two days ago, when she passed a hard stool with bright blood on the outside of it.

Tonight, her pain has worsened - after eating a meal - and she is moderately in distress. There are still no bowel motions or flatus; moreover, her bowel sounds are inaudible. She is febrile (T - 37.4 °C); in addition, there is a left shift in the blood test and her Hb was 9.3.

Please note, she had had an ovarian cystectomy and appendectomy. She was also rushed to hospital at the beginning of this month due to hemorrhage, which was diagnosed as spontaneous abortion.

I am referring her to you with the diagnosis of early bowel obstruction due to diverticulitis or carcinoma, and for further management and treatment including surgery.

Yours sincerely,

Dr X -----

Dr William Ammerly
100 Collins St,
Melbourne

22nd Feb 2006

Re: Miss Sally Webster
Aged: 17

Dear Dr Ammerly,

This 17 year old student girl first visited my surgery two months ago complaining of chronic constipation. She claimed that she had been trying everything from bran to laxatives to relieve her once every 4-5 days bowel habits, and yet, this condition still persists.

The physical examination was unremarkable (her weight was 54kg). Her request for the strongest laxative was refused; however, increased intake of fluids and vegetable fibers were recommended to improve her condition.

On her next visit six weeks later, her mother accompanied her. The mother was concerned about her daughter's loss of appetite, loss of weight, and the constant arguing at home regarding Sally's eating habits. When spoken to Sally directly, she claimed that she did not know what all the fuss was about, as she was not hungry.

On examination, she was under weight (47kg), pale and thin. Hence, several tests were prescribed, and she was asked to re-visit me at a later date for one to one assessment.

On her re-visit 6 days later, she was distant with little eye contact. She felt her parents were over-reacting, claiming her ideal weight to be 40kg (current weight 47kg). When asked, she denied vomiting and taking laxatives. After all factors considered, I believe Sally is suffering from Anorexia Nervosa.

I would appreciate if you could examine, diagnose and treat this young lady as you think appropriate.

Thank you in advance,

Yours sincerely,

Dr X -----

Dr Michaels
The Registrar,
St Paul's Hospital,
Victoria Rd,
Melbourne

Date: 20 June 2006

Re: Mrs Julie Hobart

Dear Dr Michaels,

This 33 year-old woman came to my surgery about one week ago, complaining of abnormality in her menstrual cycle. She claimed that her menstrual periods had become sparse and eventually ceased completely. She also claimed to have stopped exercising a year ago because of feeling fatigue and weakness. Meanwhile, she began to gain weight, and noticed the development of facial hair and acne.

On examination, she was mildly obese (W- 73kg, H- 1.57m) with thin extremities. There were fullness of the supraclavicular fat pads and generalised muscular weakness, as she was unable to stand from squatting position without help or difficulty standing from a seated position. In addition to a visible increase of facial hair and acne, there were marked striae on her abdomen and buttocks. While other physical examinations were unremarkable, there was a slight measured hypertension (BP 153/98).

With the possible diagnosis of Cushing's syndrome, early morning cortisol level was requested as well as FBE and Urine analysis. Meanwhile, to control the hypertension, Thiazide diuretics were prescribed.

On her visit today, she reported polyurea and nocturea. Her blood pressure was 144/99, while the result of her laboratory tests showed FBE = 136 and serum glycate = 85%. The morning cortisol level was 21µg/dl.

I am referring this patient to you with the diagnosis of Cushing's syndrome accompanied with Diabetes Mellitus. I would appreciate if you could examine and manage this patient following further tests to confirm my diagnosis.

Yours sincerely,

Dr X------(GP)

Dr James Collins
256 South Borough Lane
Brisbane
QLD 4290

Date: 21 July 2006

Re: Mrs Heather Lincoln
17 Highcombe Place
Brisbane
QLD

Dear Dr Collins,

Thank you for seeing Mrs Lincoln, an 85-year-old patient of mine who was first brought to my surgery two months ago by her daughter. She was suffering from urinary incontinence and abdominal pain for a week. The daughter claimed that her mother was more confused than usual. In addition, she had refused to eat at the time.

On examination, there was mild suprapubic tenderness by palpation, and the urine test results confirmed UTI; Amoxicillin was, therefore, prescribed for a week. Confusion was reported to have subsided a week later with the elimination of UTI.

A month later, she was brought back to my surgery, because she was found confused, loitering the streets by her neighbours. The daughter was reassured as she was very much distressed and tearful.

On July 4, her daughter reported a further increase in her vague behaviour, unsteady gait and unbalanced emotions. She was also found lying next to her bed, incontinent.

On examination, she had postural hypotension, and exhibited a general confusion. This was assumed to be due to the high dose of Aldomet; consequently, half reduced the dose.

Today, although her gait has improved but she is still confused. Her daughter exclaims that she could no longer cope with her mother's condition and is wondering if a nursing home would be a better option.

Please note, this patient has had a history of hypertension (20 years), type II diabetes (15 years), Dementia (10 years) and recurrent UTI. Her current medications are Aldomet (250mg b.d), Indocit (15mg t.d.s) and Daonit (5mg b.d).

I would appreciate if you could assess this patient and give advice to her daughter for the best possible management.

Yours sincerely,

Dr X-----

Dr Frank Adams
Neurological Ward
South Brisbane Hospital
QLD 4101

26th July 2006

Re: Mrs Phillipa King D.O.B: 18.4.38
Unit 7a, Fremantle place
Brisbane
QLD
Ph: 07 3234 3234

Dear Dr Adams,

I am referring Mrs Phillipa King, a 68-year-old woman, with a diagnosis of CVA to you for further investigation and management. She was found two days ago by a neighbour, lying on her kitchen floor conscious and stating that she had fallen two hours previously but was unable to get up by herself. Consequently, she was admitted to R.B.H yesterday.

Initial consultation with Mrs King revealed a loquacious, distractible lady who felt her main problem was the pain in her left knee which was preventing her from walking. She was also worried about her two cats.

On examination, Mrs King is an obese, large, right handed lady who sat slumped to the right on the chair with her head and eyes also leaned the same way. She showed an UMN facial droop (L) and exhibited dribbling on the same side. Her left arm was poorly positioned under the pillow. She had a left homonymous hemianopic vision as her poor eye only followed across midline left.

Please note that she has been suffering from NIDDM, Osteoarthritis in her left knee and C.O.A.D. She is a 30 pack year smoker and is currently on Ventolin and Naprosyn. Mrs King is a pensioner who lives alone (after losing her husband 3 years ago) with no children, in a housing trust unit in Brisbane. However, a neighbour does visit her twice a week for a chat and a cup of tea.

I would appreciate if you assess and manage this patient from this point on.

Yours Sincerely,

Dr X-----

Dr Jensen
Unit 40, Manor House
Ripley Street
Brisbane
QLD 4880

Date: Sep-11- 2006

Re: Mr Paul Nigels D.O.B: 9-2-72
3 Roach Street
Brisbane QLD
Tel: 0434 333444

Dear Dr Jensen,

This thirty five year-old man has come to this hospital today complaining of headaches, which has been occurring about six weeks of the year. It generally lasts about 1-2 hours each time, especially in the mornings and it worsens by straining, coughing and other stress factors, including psychological ones, which he claims causes some visual blurring. These headaches, however, are partially relieved by panadol.

On examination, there were some observable concentration drifts in speech, slightly blurred right disc as seen by Fundoscopy, and minor cerebellar ataxia. The result of the other neurological and physical examinations was unremarkable and there were no observable features of migraine found.

The possible diagnosis at this point was stress related headaches; in addition, there were some elements in the medical history, which suggested raised ICP. Hence, CT-scan was requested, and he was asked to bring his old films for comparison. A revisit was also arranged for two weeks later.

Please note that he had a major head injury because of a car accident two years ago which put him in coma for eight consecutive days in Prince Henry Hospital. There were several post-traumatic problems including amnesia, blurred vision and limb stiffness (with normal tone). I urge you to check for the list of his present medications with Caulfield Hospital.

Meanwhile, I am referring this patient to you for a second opinion and would be grateful if you could re-examine this patient. I will include the results of all the tests with this letter, and will request the CT-scans to be sent to you directly prior to the patient's visit.

Looking forward to your feed back,

Yours sincerely,

Dr X -----

Dr Robert Vaughn
34 Volturen St,
Rewanden

Date: 19 Dec. 2006

Re: Mrs Joanne White

Dear Dr Vaughn,

Mrs White, a 36 year-old mother of two, came to my surgery a week ago, reporting a two months history of fatigue, early satiety and left upper quadrant fullness. She had lost 10 pounds of body weight during this time; however, she denied fever, night sweats, nausea, vomiting and other GI problems. Her menses were normal without excessive bleeding.

On examination, her skin was moist and warm. The spleen was palpable 9 cm below the costal margin. Other physical examinations were otherwise unremarkable. Multivitamins were prescribed, but several tests were requested (FBE, ESR, LFT and peripheral blood smear). She was asked to revisit in a week's time.

On her visit today, she is still symptomatic with the results of her tests suggesting a mild leukocytosis (WBC 12K). Other abnormalities could also be seen in other blood cells. I will include the test results with this letter for your careful examination.

Please note that she does not smoke or drink, and has not had any recent exposures or travel. Her paternal grandfather did suffer from adult onset diabetes.

I believe this matter needs further specialist intervention with a possible bone marrow biopsy to help with the correct diagnosis. I would appreciate if you could treat and manage her regarding her problem.

Thanks in advance,

Yours sincerely,

Dr X -----(GP)

Dr Vaughn
34 Volturen St.
Rewanden

Date: 19th dec 2006

Reg: Mrs Joanne White

Dear Dr Vaughn,

Thank you for visiting Mrs. White, a 36 year-old patient who visited me one week ago. She complained about 2- month fatigue, early satiety, left upper quadrant fullness and 10-pound weight loss. However, she had no history of nausea, vomiting, haematochesia, haematemesis or night sweat.

On examination, her skin was moist and warm. The spleen was palpated 9cm below left costal margin; however, there was no sign of hepatomegaly, lymphadenopathy or thyromegaly. Consequently, a full set of blood tests (FBE, ESR and Peripheral blood smear) as well as biochemical tests including LFT were requested. Furthermore, multivitamins were prescribed and she was asked to come back today.

On her visit today, her problems are still persisting. There was a wide range of results obtained from the laboratory tests. For example, an increase was seen in all kinds of WBCs; however, platelet count and Hb level were normal. In addition, the vitamin B12 level was over 2000 pg/ml, but leukocyte alkaline phosphatase level had decreased.

According to her exhibited symptoms and lab-test results, I believe that her problem is a kind of hematogenous malignancy, which calls for a bone marrow biopsy for a more precise assessment.

I would appreciate if you could provide further management and treatment for this patient from this point on.

Thank you in advance,

Sincerely yours,

Dr. X------(GP)

Mr Francis Baker
Surgical Registrar,
Victoria Hospital,
Victoria Road,
Melbourne

Date: 14 February 2007

Re: Mr John Webster

Dear Dr Baker,

This 35-year-old mechanic attended my surgery two weeks ago, complaining of six-months of crampy abdominal pain, nausea and severe intermittent diarrhoea. He claimed having seven loose, foul odoured stools per day. He denied any other symptoms; however, the pruritic skin rash developed just before the diarrhoea began.

On examination, the mucous membranes were dry, and there was a diffused abdominal tenderness with no hepatosplenomegaly. Populovesicular lesions were also visible on elbows, knees and buttocks. Other physical examinations were otherwise unremarkable including bowel sounds and sphincter tone. With the possibility of gastroenteritis, diphenoxylate was prescribed and FBE, stool exam and culture were requested.

One week later, his diarrhoea was persisting, which had resulted in a loss of 9kg body weight. Although, WBC, LDH and ALP had increased, a reduction in serum albumin was noted. Prochlorperazine and megestrol were added to his drug requirements, and an abdominal CT-scan was requested. Gluten-intake restriction was also advised based on his skin rash.

Today, he claimed that his diarrhoea had improved, and his skin rash appeared to be healing. Based on the results and observations, I am inclined to celiac sprue as a possible diagnosis.

Please note that he has just stopped smoking tobacco, and drinks only occasionally (mainly beer). He does not have any considerable medical history, though his father died of lymphoma.

I am referring this patient to you for further investigations including colonoscopy and bowel biopsy to confirm my diagnosis.

Yours sincerely,

Dr X----- (GP)

The Registrar
Emergency Department
Royal Melbourne hospital
Flemington Road
Parkville 3052

Dear Doctor,

Re: Mr Derek Romano

I am writing to refer Mr Romano, a patient of mine to you. Mr Romano, is a 46-year-old and is an insurance clerk. He is married with one child who is suffering from his first episode of ischemic (or cardiac) chest pain. The patient first attended me six months ago. His risk factors include: hypertension, smoking (one packet per day), obesity, strong family history (father died of an acute myocardial infarction aged 48), and hypercholesterolemia (Total cholesterol = 6.4 mmol/L). He has no known allergies.

After persistently elevated blood pressure readings around 150/100, patient was commenced on nifedipine and this was recently increased to 20 mg twice daily. He also uses Mylanta for reflux oesophagitis. A cardiovascular examination on 23.4.97 was normal.

Today Mr Romano presented following a minimum of one hour of crushing retrosternal chest pain. He felt nauseated and sweaty with mild dyspnoea. Examination revealed a distressed and anxious man with a pulse of 64 (sinus rhythm) and blood pressure of 160/100. Crepitations were noted on chest auscultation. Electrocardiography revealed changes consistent with an inferior myocardial infarction.

Oxygen and one sublingual anginine were given, followed by intravenous morphine (2.5mg). His pain has now settled down, but I consider that he requires admission to the Coronary Care Unit for stabilization. I will telephone later to check on his condition.

Yours sincerely,

Dr X

Mr Dooley
34 Volturen St.,
Rewanden

Dear Mr. Dooley,

re: Mr Bernard smith
24 Derid Street
Farfeth

Thanks for seeing Mr. Smith, a 77 year old retired farmer. He first presented to me in November 1991 with a five year history of right hip pain. At this time he had a two month history of severe pain in the right hip which was not responding to simple analgesia. He found it difficult to bear weight and had a decreased range of movement on his right side.

Otherwise, examination was normal. X-Ray at this time showed moderate degeneration of right hip joint consistent with osteoarthritis.

I commenced him on Indomethacin 50mg t.d.s. Initially his response was good but unfortunately he suffered a further flare up in January 1992. Indomethacin was recommenced at this time but caused significant dyspepsia. I therefore commenced Mylanta and changed him on to Tilcotil 25mg two tablets daily. Unfortunately, his hip pain has continued to worsen and the change in therapy has not helped.

I would appreciate your opinion regarding his future management and would be interested to know if he would be a suitable candidate for a hip replacement.

Thanks for your opinion.

Yours sincerely,

Dr X

Mr. B.Dooly
Orthopaedic surgeon
34 Volturen St.,
Rewanden

Date: 16th Feb 1992

Re: Mr Bernard Smith

Dear Dr Dooly,

Mr Bernard Smith is a 77 year old retired farmer with a 5 year hip trouble. When he visited my surgery 3 months ago, he was limping with a 2 months severe pain in his right hip and knee that did not respond to Panadol.

On examination, he was hypertensive (BP 150/85) and had a decreased internal rotation and flexion of right hip. Osteoarthritis was confirmed with the aid of an X-ray; consequently, he was put on a course of Indomethacin tablets (50mg t.d.s).

He visited my surgery last month complaining of the flare up of his pain. The exacerbation of the pain was due to his cessation of the medication 6 weeks before. On examination the range of his right hip movement had further decreased with inability to bear weight. A new course of endomethacin (50mg t.d.s) was recommenced.

He returned again 6 days later with severe dyspepsia. He was consequently prescribed Mylanta as he showed mild epigastric tenderness, and Indomethacin was replaced with Tilcotil (25mg 2 tablets maine.).

Mr Smith has visited my office today and the examination shows little improvement in his right hip condition. I believe, his osteoarthritis is not responding to the normal treatment with the anti-inflammatory medications and he does require further specialist attention.

I am referring this patient to you for an assessment, advice and the possibility of hip replacement if you consider him a suitable candidate.

Yours sincerely,

Dr X-----